
SURGICAL PATHOLOGY:

COPY TO: [REDACTED]

Pre-Op Diagnosis

Nothing indicated

Post-Op Diagnosis

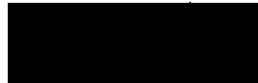
Nothing indicated

Clinical History

Nothing indicated

Gross Description:

Received in a single container labeled "[REDACTED] - right colon" is a previously opened portion of large bowel including cecum, as well as cm of previously opened portion of terminal ileum. The specimen is received after formalin fixation and shows the large bowel to measure 20.6 cm and the ileum to be 9.4 cm. The specimen is covered by a moderate amount of pericolic and epiploic adipose tissue in which an 8.0 x 0.6 x 0.6 cm grossly unremarkable vermiform appendix is identified. The visible serosa is shaggy gray tan to brown. The wall is up to 0.6 cm edematous and fibrotic. The lumen is lined by gray-tan mucosa with preservation of intestinal folds. Noted within the cecal pouch extending to the ileocecal valve is a 4.6 x 3.0 cm centrally ulcerated finely granular tan-gray to pink lesion with raised rolled borders. This is stated during specimen opening to be noted at its nearest point 26 cm from the distal margin and 14 cm from the proximal margin. This has a gritty gray tan fibrotic cut surface measuring up to 1.9 cm in thickness. This grossly extends focally through the muscularis into the surrounding adipose tissue where it is seen at its nearest point 0.7 cm from the nearest outer margin. In addition, in one area the lesion umbilicates to the serosa and grossly appears to extend through the serosa. No other gross lesions are identified. On sectioning, the surrounding adipose tissue shows several well defined and poorly defined gray tan nodules measuring up to 1.2 cm in greatest dimension. The largest nodules have fibrotic necrotic cut surfaces. Also received in the same container are three tissue cassettes each



labeled [REDACTED] Representative sections are submitted labeled as follows: A - proximal margin; B - distal margin; C - ileocecal valve; D-G - representative lesion and surrounding tissue; H - random uninvolved bowel; I - appendix; J-L - representative sections of grossly positive nodules; M-N - whole smaller pericolic nodules.

Microscopic Description:

The slides labeled [REDACTED] are examined. See diagnosis.

Final Diagnosis

Distal ileum and right colon (right hemicolectomy):

Distal ileum:

No pathologic diagnosis. PAS 1

Right colon:

Tumor characteristics:

Histologic type: Adenocarcinoma.

Location of tumor: Cecal pouch.

Size: 4.6 x 3.0 cm in greatest dimension.

Grade: Moderately differentiated.

Lymphovascular space invasion: Yes.

Perforation of Visceral peritoneum: Yes (see comment).

Presence of mesenteric deposits: No.

Depth of invasion: Carcinoma extends through muscularis propria into subserosal adipose tissue and focally perforates visceral peritoneum to involve serosal surface focally.

Surgical Margin Status:

Proximal Margin: No carcinoma identified.

Distal Margin: No carcinoma identified.

Deep Radial Margin: Carcinoma focally perforated visceral peritoneum to involve serosal surface focally (see comment).

Distance of tumor from closet margin: See above diagnoses.

Lymph Node Status:

Total number of lymph nodes examined: 17

Total number of lymph nodes with metastatic carcinoma: 11
(11/17) PAS 9

Appendix:

Fibrous obliteration of the luminal tip, no carcinoma identified.
PAS 4 SPC-A

CPT: 88309

Stage: pT4aN2bVL (see comment)

Comments

Grossly in one area, the lesion umbilicated to the serosa and grossly appears to extend through the serosa. Microscopically, carcinoma is present focally extending through the visceral peritoneum to focally involve the serosal surface. Therefore, this is staged as pT4a. Eleven of 17 lymph nodes demonstrate metastatic carcinoma. Some of the nodes are completely replaced by carcinoma with no residual nodal tissue identified. These are counted as lymph nodes totally replaced by tumor. Therefore, this is staged as a pN2b. In addition, lymphovascular space invasion is identified and the additional classifications of VL are therefore used. The final stage should be performed in conjunction with knowledge of the clinical stage.

At the request of the undersigned pathologist, these slides have



been additionally reviewed by Dr. [REDACTED] who concurs with the diagnosis.

[REDACTED]

[REDACTED]

[REDACTED]